

Family Medicine Specialists, P.C.

500 8th Avenue SE, Cedar Rapids , IA 52401

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PATIENT: Ole Mother Hubbard
 DATE OF BIRTH: 02/08/1935
 DATE: 05/17/2019 01:00 PM
 VISIT TYPE: Office Visit

This 84 year old female presents for Pre-op.

History of Present Illness:

1. Pre-op

Pt is here per the request of Dr Munjal prior to planned procedure. She has RA, has severe OA in her left knee and is going to have a joint replacement.

Her risk screening is positive for hypertension, rheumatoid arthritis. Pressures are well controlled on current meds and after renal artery stenosis treatment. Her RA is well controlled on current medications too. She has mitral valve insufficiency. She has just seen cardiology last month in anticipation of her upcoming procedure, she had a normal perfusion stress test (done because she couldn't exercise or do at least 4 mets due to leg pain). They have no cardiac concerns in relation to her upcoming surgery. She has not had reactions to anesthesia in the past.

Intake Comments: Left total knee on June 7th at St Luke's with Dr Munjal

Problem List:

Problem Description	Onset Date	Chronic	Notes
Mitral valve insufficiency	04/13/2011	Y	Mitral valve regurgitation,396.3 mild-moderate 4/2011- Hodgman.
Benign essential hypertension	08/01/2011	Y	Hypertension, benign essential,401.1 2005 meds started.
Rheumatoid arthritis	08/01/2011	Y	RHEUMATOID ARTHRITIS,714.0 1992 - Dr Brooks.
Osteopenia	08/01/2011	Y	Osteopenia,733.90
Benign essential tremor	01/21/2018	N	1/2018 - L hand

PAST MEDICAL/SURGICAL HISTORY (Detailed)

Disease/disorder	Onset Date	Management	Date	Comments
	11/2012	left knee cap fracture		
	1960	hemorrhoidectomy		
	03/2001	vaginal hysterectomy with lap bladder suspension, ovaries removed		
	06/2003	colonoscopy - hemorrhoid, diverticulosis - repeat 5 years		
	10/2008	colonoscopy - negative		

	11/11/2013	(repeat 5 yrs) colonoscopy - negative aside from hemorrhoids (no repeat needed)
pneumonia	1975	Hospitalization
pneumothorax	1970	Hospitalization
renal artery stenosis	08/22/2012	R renal artery stent placed
scarlet fever		Hospitalization

GYNECOLOGIC HISTORY:
Patient is postmenopausal.

Family History (Detailed)

Relationship	Family Member Name	Deceased	Age at Death	Condition	Onset Age	Cause of Death
Father		Y	78	Congestive heart failure		Y
Father		N		Cancer, breast		N
Father		N		Hypertension		N
Grandson		Y	20	Leukemia (died from stroke)		Y
Mother		Y	83	Congestive heart failure		Y
Mother		N		Stroke		N
Son		N		CAD (s/p CABG x 5)		N

SOCIAL HISTORY (Detailed)

Preferred language is English.

EDUCATION/EMPLOYMENT/OCCUPATION

Employment	History	Status	Retired	Restrictions
Mercy Medical Center	gift shop employee/ volunteer			

Smoking status: Former smoker.

SMOKING STATUS

Use Status	Type	Smoking Status	Usage Per Day	Years Used	Total Pack Years
yes	Cigarette	Former smoker	0.5 Packs	16.00	8.00

CESSATION

Type	Date Quit	Longest Tobacco Free	Cessation Method
Cigarette	01/01/1966		

Medication Reconciliation

Medications reconciled today.

Medication Reviewed

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Medication Name	Agnerence	Sig Desc	Where	Status
taking as directed	Multivitamin Tab	Take one by mouth daily	N	Verified
taking as directed	Os-Cal 500 + D 500 mg (1,250 mg)-200 unit Tab	As Directed	N	Verified
taking as directed	Cozaar 100 mg Tab	take 1 tablet (100MG) by oral route every day	N	Verified
taking as directed	folic acid 1 mg tablet	take 3 tablets daily	N	Verified
taking as directed	amlodipine 10 mg tablet	take 1 tablet by oral route every day	N	Verified
taking as directed	meloxicam 15 mg tablet	take 1 tablet by oral route every day	N	Verified
taking as directed	methotrexate sodium 2.5 mg tablet	Take 6 pills weekly	Y	Verified
taking as directed	Prolia 60 mg/mL subcutaneous syringe	inject 1 milliliter by subcutaneous route every 6 months in the upper arm, upper thigh or abdomen	Y	Verified

Allergies

Ingredient	Reaction	Medication Name	Comment
MEPERIDINE HCL	Nausea	DEMEROL	
LISINOPRIL	Cough		

Reviewed, no changes.

REVIEW OF SYSTEMS

System	Neg/Pos	Details
Neuro	Positive	Gait disturbance.
Reproductive	Positive	The patient is post-menopausal.
MS	Positive	Joint pain, Joint swelling.
Eyes	Negative	Eye discharge and vision loss.
GI	Negative	Abdominal pain, change in stool pattern, constipation, diarrhea and vomiting.
Respiratory	Negative	Cough, dyspnea and wheezing.
Neuro	Negative	Headache.
GU	Negative	Dysuria and hematuria.
ENMT	Negative	Ear drainage, nasal drainage and sinus pressure.
MS	Negative	Back pain and muscle weakness.
Integumentary	Negative	Pruritus and rash.
Cardio	Negative	Chest pain, claudication, edema and irregular heartbeat/palpitations.
Reproductive	Negative	Vaginal discharge.
Hema/Lymph	Negative	Easy bleeding, easy bruising and lymphadenopathy.
Constitutional	Negative	Fatigue, fever and malaise.
Cardio	Comments	No trouble with BP med. EKG 5/14/2018 normal.
Hema/Lymph	Comments	Normal labs including CBC, CMP, UA, staph cultures.

VITAL SIGNS

Time	BP	Pulse	Resp	Temp	Ht ft	Ht in	Ht cm	Wt lb	Wt kg	BMI	BSA	O2 Sat%
	mm/Hg	/min	/min	F						kg/m2	m2	
1:08 PM	130/60	72	12	97.9		61.50	156.21	112.00	50.802	20.82		96

MEASURED BY

Time	Measured by
1:08 PM	Erin Brown, CMA

Physi-

Exam

Findings	Details
Constitutional	Normal Well developed.
Eyes	Normal Conjunctiva - Right: Normal, Left: Normal.
Ears	Normal Canal - Right: Normal, Left: Normal. TM - Right: Normal, Left: Normal.
Nasopharynx	Normal Lips/teeth/gums - Normal. Oropharynx - Normal.
Neck Exam	Normal Inspection - Normal. Palpation - Normal. Thyroid gland - Normal.
Lymph Detail	Normal No cervical or supraclavicular adenopathy.
Respiratory	Normal Auscultation - Normal. Effort - Normal.
Cardiovascular	Normal Regular rate and rhythm. No murmurs, gallops, or rubs.
Vascular	Normal Capillary refill - Less than 2 seconds.
Abdomen	Normal Inspection - Normal. No abdominal tenderness. No hepatic enlargement. No splenic enlargement.
Skin	Comments no rashes
Musculoskeletal	Comments left knee decreased ROM
Extremity	Normal No edema.
Psychiatric	Normal Orientation - Oriented to time, place, person & situation. Appropriate mood and affect.

Assessment/Plan

#	Detail Type	Description
1.	Assessment	Rheumatoid arthritis, unspecified (M06.9).
2.	Assessment	Primary osteoarthritis of left knee (M17.12).
3.	Assessment	Preoperative cardiovascular examination (Z01.810).
	Patient Plan	Pt has been medically optimized in anticipation of her upcoming procedure. Rheumatology aware and will be managing her meds around the surgery. Cardiology has also given clearance for procedure. Remainder of plan of care per Dr Munjal and ortho staff.

Medications (Added, Continued or Stopped this visit)

Started	Medication	Directions	Instruction	Stopped
09/29/2016	amlodipine 10 mg tablet	take 1 tablet by oral route every day		
09/05/2013	Cozaar 100 mg Tab	take 1 tablet (100MG) by oral route every day		
09/24/2015	folic acid 1 mg tablet	take 3 tablets daily		
09/29/2018	meloxicam 15 mg tablet	take 1 tablet by oral route every day		
09/29/2018	methotrexate sodium 2.5 mg tablet	Take 6 pills weekly		
09/05/2008	Multivitamin Tab	Take one by mouth daily		
09/05/2008	Os-Cal 500 + D 500 mg (1,250 mg)-200 unit Tab	As Directed		
09/29/2016	Prolia 60 mg/mL	inject 1 milliliter by		

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subcutaneous
syringe

subcutaneous route every 6
months in the upper arm,
upper thigh or abdomen

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